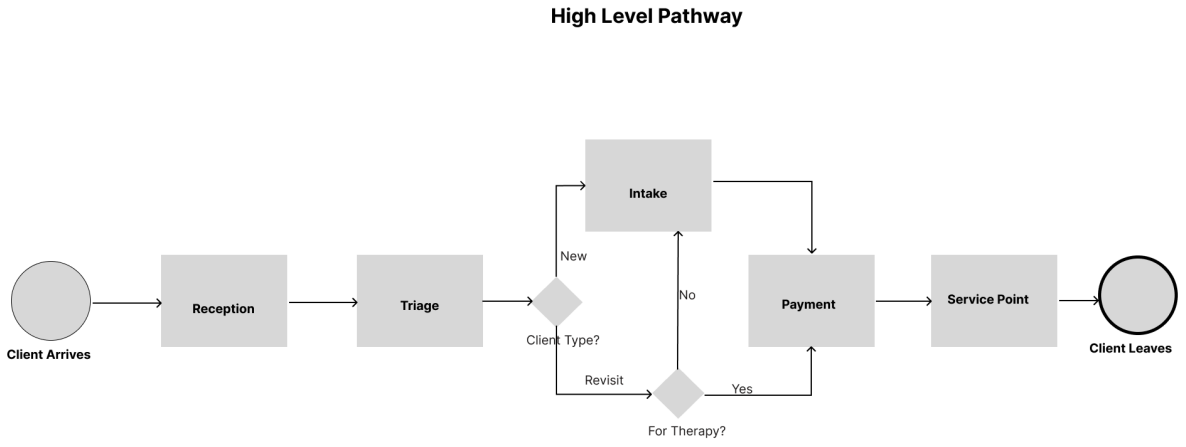
High-Level Client Pathway – Description

The High-Level Pathway illustrates the complete movement of a client through KISE, from arrival to registration, assessment, service delivery, and exit.



1. Client Arrival

- The process starts when a client arrives at the facility.
- The client is received at **Reception** for registration or verification.

2. Reception and Check-In

- The receptionist confirms whether the client is **new** or **returning**.
    - **New clients** are registered in the HMIS and automatically assigned a **KISE Number (UCI)**.
    - **Returning clients** are searched and signed in using their existing KISE Number.
- The receptionist performs **check-in**, which captures:
    - Client arrival time
    - Service type or purpose of visit (for clients who were previously booked)
- The **check-in timestamp** marks the official start of the client's visit and enables later computation of total time spent in the facility.
- After check-in, the client proceeds to **Triage**.

3. Triage

- The triage clinician conducts an initial screening to establish the client's health and service needs.
- Clients are categorized as high risk, medium risk and low risk:
- Those with low risk (or medium/high risk once stabilized) are routed as follows:
    - **New:** Proceed to **Intake** for baseline assessment.
    - **Revisit:** Proceed directly to **Payment** if returning for therapy or follow-up.

4. Intake

- Intake officers collect client background information, verify NCPWD number, conducts general assessment to guide sign posting of clients and confirm service availability.
- All information is reviewed and updated in the HMIS against the KISE Number.
- The client is then referred to **Payment** for billing.

5. Payment

- The payment officer/SHA officer registers the eligible clients under the Social Health Authority (SHA) insurance scheme.
- The cashier officer verifies and records the selected payment method (SHA insurance, or e-citizen, NCPWD etc).
- Payment information is logged in the HMIS and linked to the client's visit record.
- Upon completion, the client proceeds to the assigned **Service Point**.

6. Service Point

- The client receives the required service (e.g., assessment, therapy session, or assistive technology, etc).
- Providers document interventions, outcomes, and follow-up plans in the HMIS.
- If a follow-up is required, an appointment is scheduled.
- If no further service is needed, the visit is marked ready for **check-out**.

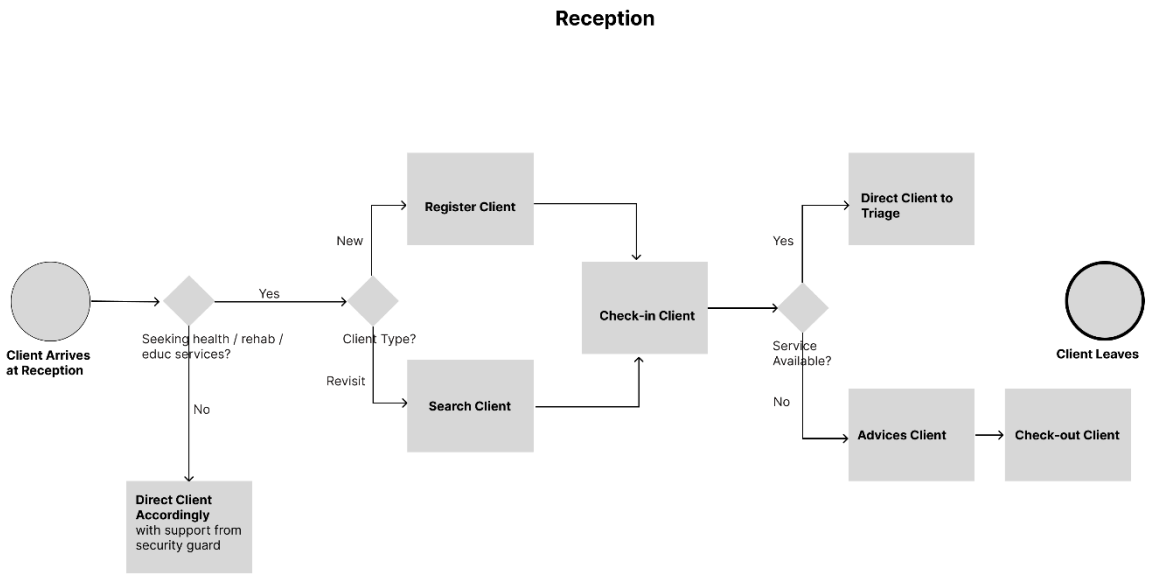
7. Client Check-Out and Exit

- At the end of the final service, the client is formally **checked out** of the HMIS.
- **Check-out** captures the departure time and confirms that all required documentation, services, and payments are complete.

- The difference between **check-in** and **check-out** timestamps provides the **total client time in the facility**, which supports workflow optimization and efficiency monitoring.
- The visit is closed in the system with a “Completed” status, and the client exits.

Main Reception Workflow

Manages client registration as they enter into KISE. Ensures accurate identification, service eligibility confirmation, and smooth routing to triage or intake.



1. Client Arrival and Identification

- The client arrives at the **Reception** desk.
- The receptionist determines whether the client is:
    - **Previously Registered:** The client already has a record in the KISE HMIS (either from KISE–Kasarani or a satellite/outreach site).
    - **New Client:** No existing record in the HMIS.
- The receptionist searches the HMIS using the **KISE Card**.
- If the card is unavailable, the receptionist may search the HMIS using **client name** or **phone number**, then confirm the client’s identity before proceeding.

2. Registration or Record Retrieval

- **Previously Registered Clients:**
  - The receptionist retrieves the existing profile.
  - Demographic details are verified.
- **New Clients:**
  - The receptionist completes the **Client Registration Form**, capturing four essential data fields:
    - Client/child name
    - Guardian's name (if applicable)
    - Phone number
    - Sex and Date of Birth
  - The HMIS creates a new profile and automatically assigns a **KISE Number (UCI)**.

3. Sign-In (Check-In) Process

- The receptionist clicks the **Sign In** button on the client's HMIS profile.
- The HMIS automatically records the **date**, **time**, and **receptionist ID**, confirming the client's official arrival for services.
- The **check-in timestamp** marks the start of the client's visit and is later used to compute the total duration in the facility.

4. Service Availability Check

- The receptionist verifies whether the requested service is available that day.
  - **If services are available:** The client is directed to **Triage** for initial screening.
  - **If services are not available:** The client is advised accordingly (e.g., rescheduled or referred).
    - The receptionist performs a **Check-Out** in the HMIS, closing the visit.
- For **returning clients** who were already assigned a specific service:
  - Once the service availability is confirmed, the client is **signed in**, and the system **auto-generates a billing entry** linked to their visit.

Notes

- Service availability data used at Reception is updated daily by Customer Care at each service point, as outlined in the *Customer Care – Service Availability and Notification Workflow*.
- Receptionists operate under limited HMIS access, restricted to **Registration**, **Sign-In**, and **Check-Out** modules.
- All records created or retrieved at Reception are immediately accessible to other departments (Triage, Intake, Therapy, etc.) according to user permissions.

Customer Care at the blocks – Service Availability and Client Notification Workflow

Handles daily updates of service availability across all service points.
Automatically notifies booked clients, individually or in groups, through SMS when service changes occur.

1. Daily Service Status Update

- Each **Customer Care Officer** updates the service availability status for their service block **at the end of the day or before 7:00 AM**.
- They mark each service as **Available** or **Unavailable**.
- If *Unavailable*, they must:
    - Select a **reason code** (e.g., staff unavailable, equipment fault, or system downtime).
    - Add a short **comment** explaining the issue.

2. Identify Affected Clients

- The HMIS automatically lists all **clients booked for the next day** under the affected service point.
- Customer Care can review the list and **group clients** with the same service or appointment type.

3. Notify Clients

- The officer triggers **group notifications** via the integrated **Celcom SMS gateway**.
- The system allows two types of messages:
    - **Service Confirmation SMS:** sent to confirm that the booked service remains available.

- **Service Disruption SMS:** sent when a service is unavailable, advising clients to reschedule or await further communication.

- Group messages are preferred where possible to reduce SMS volume and cost.

4. Synchronization Across Modules

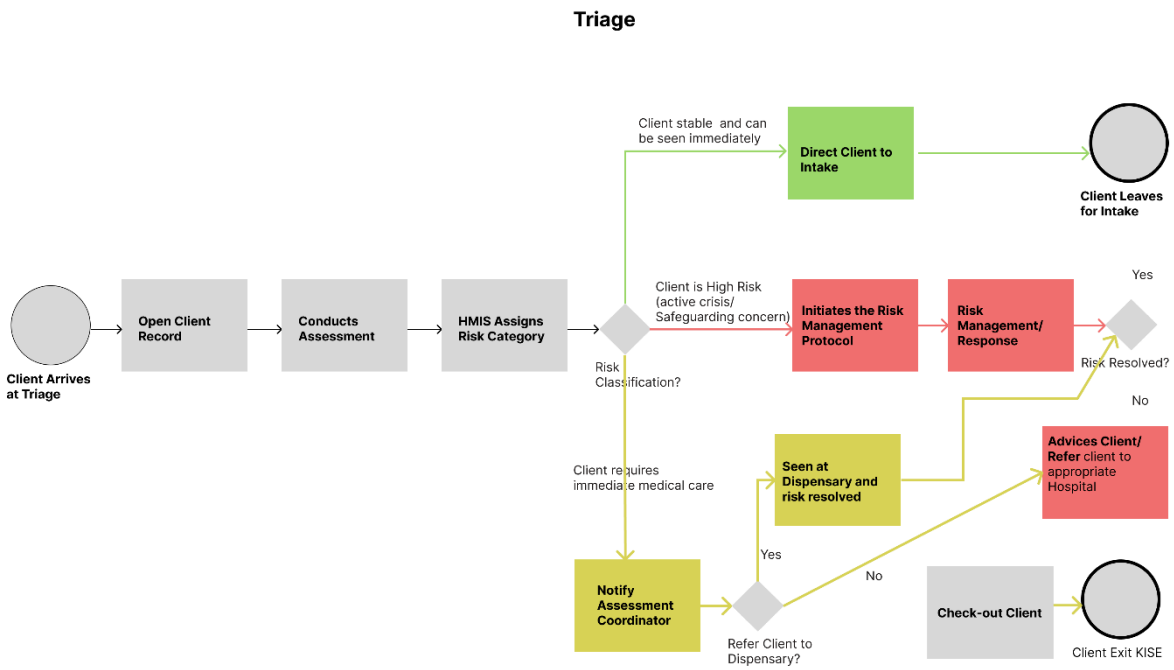
- Any change in availability automatically updates:
  - **Reception and Booking modules**, to prevent new bookings in closed service blocks.
  - **Reports and dashboards**, to track service disruptions and communication patterns.

5. Accountability

- All updates and notifications are logged with **timestamp** and **staff ID** for audit and performance tracking.

Triage Workflow

Every client arriving from Reception must undergo triage before any service.
This step ensures safety through standardized screening, automated risk scoring, and controlled routing to the next stage.



1. Client Arrival at Triage

- The client, already checked in at Reception, automatically appears in the **Triage queue** on the HMIS.
- The **Triage Clinician** opens the client record, verifies demographic data, and reviews the reason for visit.

2. Triage Screening (Mandatory Step)

- The clinician completes the **Standardized Triage Screening Tool** to assess medical status, mental wellbeing, and immediate risks.
- **System logic:** The HMIS automatically calculates a **Risk Score** and assigns a **Risk Flag** — *Low, Medium,* or *High*.
- This screening is a **mandatory safety gate** and must be completed before any service assignment or assessment booking.

Path A – Standard Flow

Condition: Client is stable (Low or Medium risk) and fit to proceed for services.
Action:

- The clinician records findings in the HMIS.
- Routing logic:
  - **New clients** (e.g., educational assessment, placement, medical report review, or first-time therapy) → directed to **Intake**.
  - **Returning clients** (confirming therapy) → directed to **Payment/Cashier**.
- **System action:** HMIS updates the client status to *"Cleared for Assessment."*

Path B – Medical Hold

Condition: Client has unstable vitals, injury, illness, or requires immediate medical attention.
Action:

- The clinician refers the client to the **Dispensary**.
- The triage officer alerts the **Assessment Coordinator**.
- The coordinator informs the client/caregiver, explains the reason for delay, and **reschedules** once medically cleared.
- **System action:** Client record flagged *"Medical Hold"* until clearance.

Path C – Crisis Protocol

Condition: Client presents with a **High Risk flag** (active crisis or safeguarding concern such as self-harm, violence, or abuse).
Action:

- The clinician activates the **Crisis Management Protocol** immediately.
- The triage officer alerts the **Assessment Coordinator**, who mobilizes the appropriate response (counsellor, safeguarding focal person, security, or crisis response team).
- The coordinator oversees the entire response and documentation.
- **System action:** Client record tagged *"Crisis – Protocol Initiated."*

3. Documentation and Handover

- All triage findings, risk categories, and actions are recorded in the HMIS.
- Once cleared (from any path), the client proceeds to **Intake** for assessment scheduling or the next appropriate step.

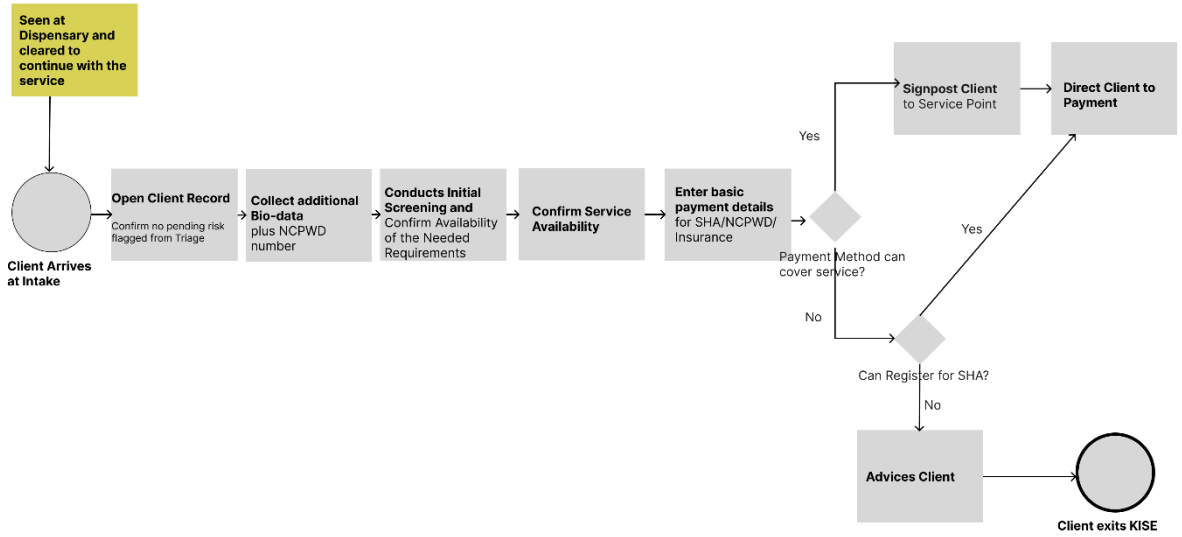
Key Points

- **Mandatory process:** No client may bypass triage screening.
- **Automated risk scoring:** Promotes uniformity and rapid decision-making.
- **Defined roles:**
    - *Triage Clinician* — conducts screening and categorization.
    - *Assessment Coordinator* — manages escalations, client communication, and rescheduling.
- **System integration:** Risk flags and status updates are instantly visible to all relevant service points.

Intake Workflow

The Intake process confirms client eligibility, collects additional details, and directs the client to the appropriate service point.
It ensures that every client is correctly linked to an available service and an approved payment method.

Intake



1. Client Arrival at Intake

- The client arrives at the **Intake desk**, automatically appearing in the HMIS queue after being cleared from Triage or the Dispensary.
- The **Intake Officer** opens the client's record and confirms that there are **no pending risk flags** from previous stages.

2. Client Verification and Data Collection

- The officer collects **additional bio-data**, including:
  - NCPWD Number (if available),
  - Contact details, and
  - Any missing demographic information.
- The HMIS updates these details in the client's record under the assigned **KISE Number (UCI)**.

3. Initial Screening and Service Confirmation

- The officer conducts an **initial screening** to understand the **main service** the client is seeking and to determine which **service point** they should be posted to.
- The officer then verifies the **availability of required services and resources**, such as therapists, assessors, or assistive technology.

- The screening also confirms whether any **pre-requisite documents or assessments** are available (e.g., referral letters, prior medical reports).
- **System logic:** The HMIS records this verification as part of the **Intake Checklist** and makes it visible to subsequent service points.

4. Payment and Eligibility Review

- The officer enters **basic payment details** for the selected payment method — *SHA, NCPWD, insurance, or self-pay.*
- The system evaluates whether the payment method can cover the intended service.
    - **If covered:**
        - The client is **signposted to the relevant service point** and directed to **Payment** for billing confirmation.
    - **If not covered:**
        - The officer checks whether the client qualifies for registration under the **SHA scheme.**
            - **If eligible:** The officer refers the client to be assisted with **SHA registration** by the **SHA Officer.**
            - **If ineligible:** The client is **advised accordingly**, and the visit is **closed** in the HMIS with a *"Service Deferred"* status.

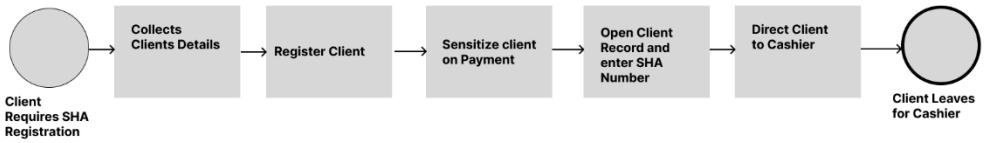
5. Documentation and System Actions

- Each action — data collection, service screening, payment entry, and eligibility outcome — is timestamped and recorded in the HMIS.
- The client's updated details, payment status, and service assignment become instantly visible to **Payment** and the **Service Point**.

SHA / Insurance Registration Officer Workflow

This process facilitates registration and verification of clients under the Social Health Authority (SHA) or other supported insurance schemes. It ensures that all eligible clients are correctly enrolled, linked to their payment records, and guided to the cashier for billing.

SHA Registration Officer



1. Client Referral for Registration

- The client is referred to the **SHA Registration Officer** from **Intake** when the selected payment method cannot cover the required service.
- The officer confirms the client's need for registration and checks for existing SHA or insurance records to prevent duplication.

2. Client Information Collection

- The officer collects necessary **client details**, including:
  - Full name and contact information.
  - National ID or birth certificate number.
  - NCPWD number (if applicable).
  - Service type for which the insurance will apply.
- The details are entered into the **SHA registration portal or insurance system** for account creation.

3. Registration and Account Creation

- The officer completes the **client registration process** in the SHA or insurance system.
- Upon successful registration, the client is issued a **unique insurance membership number** (e.g., SHA number).
- This number is used to link the client's insurance account with their **KISE HMIS record**.

4. Sensitization and Financial Guidance

- The officer provides brief **sensitization on payment coverage**, explaining:
    - Which services are covered under the scheme.
    - Any co-payments or exclusions that may apply.
    - Expected turnaround times for claim validation.
- This ensures the client and caregiver understand their coverage before proceeding to payment.

5. Record Update and Referral to Cashier

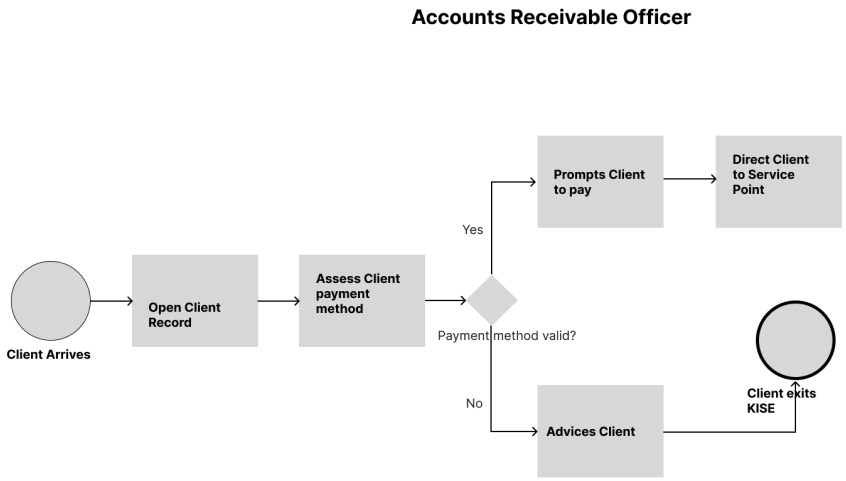
- The officer **opens the client's HMIS record** and updates it with the assigned **SHA/Insurance Number.**
- The HMIS automatically links the insurance details to the visit and marks the client as **"Insurance Verified."**
- The client is then **directed to the Cashier** for billing and service payment confirmation.

6. Documentation and System Integration

- All registration actions are timestamped and recorded with the staff ID of the SHA/Insurance Officer.
- The system syncs registration data across **Payment, Intake, and Service Point modules** for visibility and audit tracking.
- **Scalable design:** While currently focused on SHA, the workflow supports future inclusion of additional schemes (NHIF, private insurers, etc.).
- **Client experience:** Ensures no eligible client is turned away due to lack of coverage and reduces manual delays during billing.

Accounts Receivable (Cashier) Workflow

This process manages client billing, payment confirmation, and official receipt issuance before service delivery. It ensures all payments e.g. insurance, or eCitizen, are accurately captured and linked to the client's HMIS visit record.



1. Client Arrival at the Cashier

- The client arrives at the **Accounts Receivable desk** after clearance from **Intake** or **SHA/Insurance Registration**.
- The **Cashier Officer** opens the client's record in the HMIS to verify that the visit is active and payment details are complete.

2. Payment Method Assessment

- The cashier reviews the **selected payment method**, which may include:
  - **Insurance schemes** (e.g., SHA, NCPWD, or private insurers).
  - **eCitizen payments** (made via M-PESA and synced to HMIS).
- The system automatically displays the selected payment method and checks its **validity and coverage** for the requested service.
  - **If the payment method is valid:**
    - The client is prompted to pay through the appropriate channel.
    - Once payment is verified, the **HMIS generates a system receipt**.
    - The cashier then **prints and issues the official receipt** to the client or caregiver.
    - The HMIS logs the receipt number, amount paid, payment channel, and officer ID.

        - The client is then **directed to the Service Point** for service delivery.
    - **If the payment method is invalid or incomplete:**
        - The cashier explains the issue (e.g., inactive coverage, failed eCitizen confirmation, or insufficient balance).
        - The client is **advised accordingly**, and the visit is closed with a *"Payment Deferred"* or *"Service Deferred"* status.

3. Receipt Editing and Adjustment

- If the **service offered changes after payment** (e.g., substitution of a therapy session or cancellation):
    - The cashier accesses the existing payment record in the HMIS.
    - Makes the authorized adjustment.
    - **Generates and prints a revised receipt** clearly marked as an "Adjusted Receipt."
- All modifications are logged in the system with:
    - User ID,
    - Date and time of change, and
    - Justification note for audit tracking.

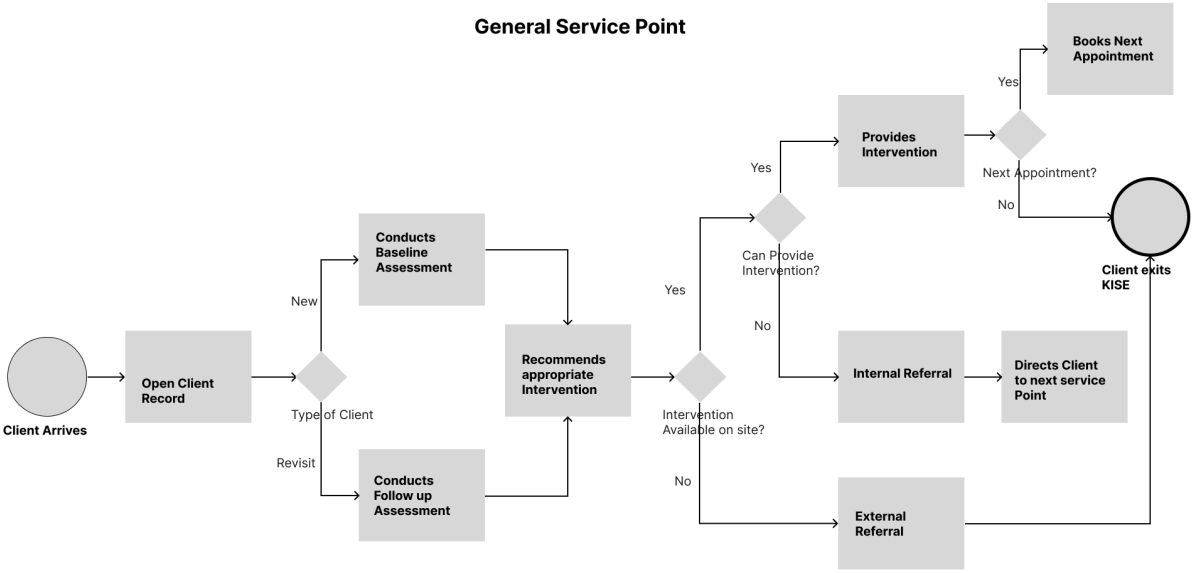
4. System Documentation

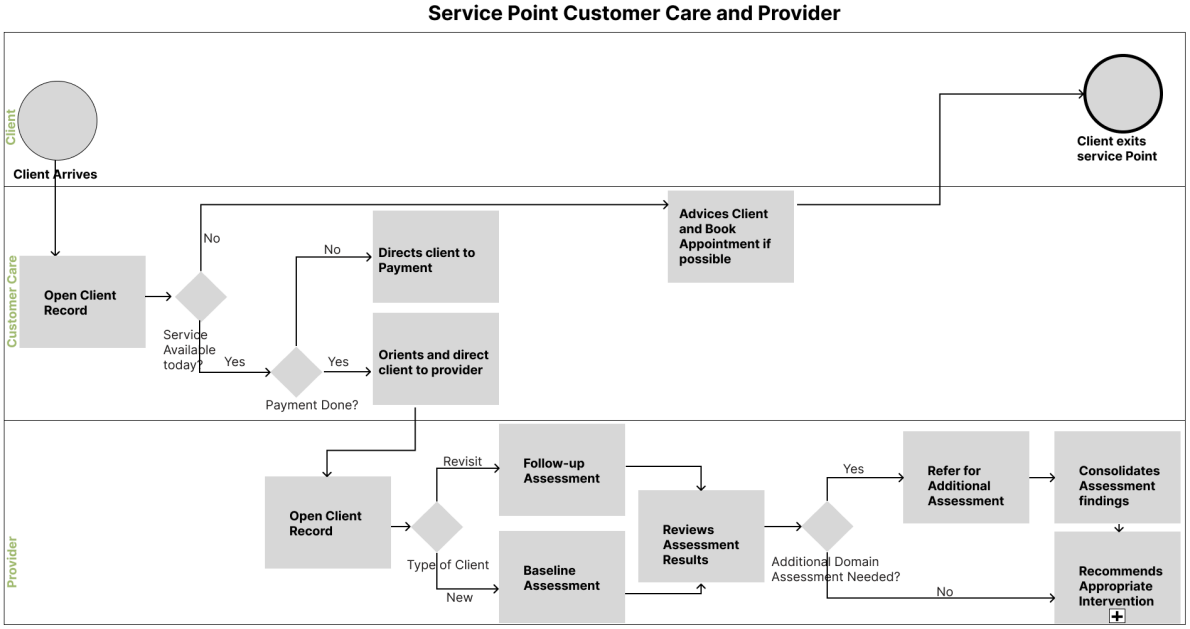
- All payment records—original or adjusted—are time-stamped, linked to the visit, and synced with the **Finance and Reporting modules**.
- The cashier can retrieve or reprint receipts as needed for audit, client request, or reconciliation.

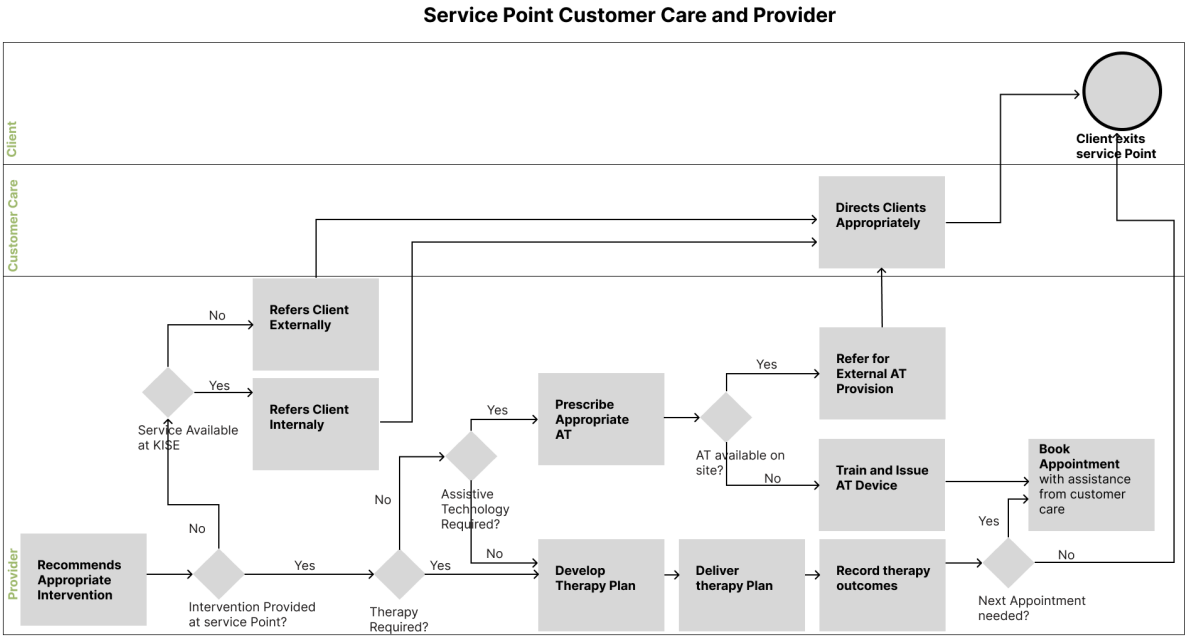
General Workflow for service points

This workflow captures how clients receive assessments, therapy, or assistive technology services at each service point. It integrates provider decision-making, customer care coordination, and system documentation for complete service tracking.

General Service Point







1. Client Arrival and Pre-Service Support (Customer Care)

- A client arrives at the **service point** (therapy, assessment, or assistive technology).
- **Customer Care** searches for the client in the HMIS:
  - If found → the client proceeds.
  - If not found → the client is redirected to **Reception** for registration.
- Customer Care verifies:

    - The **purpose of visit**,
    - **Booking and payment status**, and
    - **Service availability** (based on updates from each block).
- If services are available → the client is oriented and guided to the assigned professional.
- If unavailable → the client is advised, and the visit ends with a *"Deferred – Service Unavailable"* status.

2. Assessment and Initial Decision-Making (Service Provider)

- The provider opens the client's HMIS record and confirms identity, visit type, and referral reason.
- The system distinguishes between:
    - **New clients:** Conduct a **baseline assessment**.
    - **Returning clients:** Conduct a **follow-up assessment**.
- Based on assessment findings, the provider **recommends an appropriate intervention**:
    - Therapy (speech, occupational, physical, etc.)
    - Assistive technology (prescription, fitting, training)
    - Referral (internal or external)
- The provider confirms whether the intervention can be provided **onsite** or requires input from another service domain.

3. Cross-Domain Assessment (If Required)

- If another domain's input is required:
    - The provider raises an **internal referral** in HMIS (e.g., from educational assessment to audiology).
    - The referral appears on the receiving department's dashboard.
    - Customer Care coordinates and schedules the next available slot.
- Each domain conducts its own assessment and records findings in the **shared client record**.
- When all assessments are completed, results are reviewed collectively during a **case conference** to finalize the intervention plan.

4. Intervention Delivery

A. Therapy Pathway

- The provider develops a therapy plan and conducts sessions as scheduled.
- Progress, goals, and attendance are recorded after each session.
- If continued therapy is required:
  - The provider books the **next appointment** and sets reminders through HMIS.
  - The client is guided to **Customer Care** for confirmation.

B. Assistive Technology (AT) Pathway

- The provider prescribes an assistive device and checks its availability.
  - **If available:** Device is issued, and client/caregiver trained.
  - **If unavailable:** The provider makes an internal or external referral.
- The device type, serial number, fitting details, and training notes are logged in the HMIS.

5. Intervention Not Available Onsite

- When the required service or intervention cannot be provided:
  - The provider initiates an **internal referral** to another KISE service point, or
  - An **external referral** to a partner facility.
- Referral details (reason, contact, next appointment date) are recorded in HMIS.
- Customer Care assists in notifying and tracking referred clients.

6. Post-Service Engagement (Customer Care)

- Once services or referrals are completed, Customer Care:
  - Confirms completion with the provider.
  - Updates the client's visit record and ensures it's closed or scheduled for follow-up.
- If a **follow-up appointment** is needed:
  - It is booked in HMIS, and an **SMS reminder** is scheduled through the integrated notification system.
- Customer Care also collects **client feedback or complaints**, escalating cases as needed.

- The visit is formally **checked out**, marking the end of the client's journey for that service.

Customer Care Workflow (Stationed at Service Point)

This process ensures clients receive clear guidance, timely updates, and administrative support at the service point.
Customer Care bridges clients, providers, and the HMIS—enhancing flow, feedback collection, and service visibility.

1. Client Arrival at Service Point

- Customer Care greets the client and verifies the **purpose of visit** (assessment, therapy, follow-up, or referral).
- Searches the client's **record in HMIS** to confirm the scheduled service and assigned provider.
- If the client record is missing, directs them to **Reception** for registration before rejoining the queue.

2. Pre-Service Support (Before Assessment/Therapy)

- Provides **orientation** on the process, expected waiting time, and next steps.
- Addresses **general inquiries** or clarifies previous instructions (e.g., missing documents, preparation for therapy).
- Where necessary, assists in **confirming payment status** or resolving simple administrative issues to prevent delays.
- Flags any **special needs** (mobility, communication, or caregiver support) in the HMIS and alerts the service provider.

3. During Service (Monitoring and Coordination)

- Monitors the client flow to ensure that clients are seen in proper order.
- Provides **real-time updates** to waiting clients if delays occur.
- Coordinates with providers to manage scheduling changes or unexpected absences.
- Maintains a calm, organized waiting area and addresses emerging client concerns.

4. Post-Service Engagement (After Assessment/Therapy)

- Confirms with the provider that the session or service is complete.

- Could assist in updating the client's **visit record** in HMIS with session completion details.
- If a follow-up is required:
    - Schedules or confirms the **next appointment** in HMIS.
    - Sends an **SMS reminder** where integration allows.
- Provides **referral guidance** for clients directed to another KISE service point or external facility.
- Collects **feedback** through approved platforms
- Logs and escalates any **complaints** to the designated supervisor for resolution.

5. Visit Closure

- Ensures the client's visit record is **checked out** in the HMIS once all actions are complete.
- Provides any takeaway materials—therapy schedule, referral note, or next visit slip.
- Politely directs the client to the **exit or next service point**, ensuring proper handover if applicable.

Key Points

- **System linkage:** Customer Care uses the HMIS to track check-in, completion, and follow-up.
- **Efficiency role:** Keeps clients informed and reduces waiting-time anxiety.
- **Quality input:** Captures feedback that informs service improvement.
- **Collaboration:** Works closely with providers and reception to ensure continuity of care.